The Hillingdon Hospitals
NHS Foundation Trust

Prevention and Management of Pressure Ulcers in Maternity

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Version	Author	Summary of changes
1.0	Gillian Pearce Maternity Governance Midwife	New guideline as a result of incidents discussed with TVN
2.0	Daisy Culaj Lead TVNS Gillian Pearce Maternity Governance Midwife	Update to guideline as a result of incident Change of risk assessment tool and process

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Aim/Purpose

The aim of this guideline is to identify women/birthing people antenatally, intrapartum and postnatally who are at risk of developing a pressure ulcer. The guideline identifies the differences between moisture associated skin damage (MASD), pressure ulcers/injuries and subsequent management.

Introduction

A pressure ulcer can affect any person of any age, it is a localised injury to the skin and/or underlying tissue, commonly over a bony prominence, as a result of sustained pressure, or pressure in combination with shear. Pressure ulcers can cause pain and may prolong inpatient admissions.

NICE guidance *Preventing Pressure Ulcers in Adults* requires acute trusts to offer adults at risk of developing a pressure ulcer a skin assessment and a care plan for women/birthing people who have been subsequently identified as high risk. It is important to assess all women/birthing people on admission to the unit. Women/birthing people with an increased BMI (more than 40), or an increased BMI (more than 35) with additional comorbidities, significant limited mobility (for example, spinal cord injury), significant loss of sensation, a previous or current pressure ulcer(s), nutritional deficiency, the inability to reposition themselves or significant cognitive impairment. It is important to note the women/birthing people's level of risk may fluctuate during their admission.

Explanation of terms used in policy

Purpose T	PURPOSE-T interprets pressure ulcer risk factor evidence and expert opinion into a practical clinical tool that can assist with the identification and management of pressure ulcer risk in practice
Eschar	Dead tissue that ejects from healthy skin. It is caused by a burn or cauterization (destroying tissue with heat or cold, or another method). An escharotic is a substance (such as acids, alkalis, carbon dioxide, or metallic salts) that causes the tissue to die and fall off
Slough	Yellow/ off white deposit in the wound base, usually wet but can be hard and dry, generally has a soft texture. It can be dense and adhered to the wound base, presents as a thin covering or patchy over the surface of the wound. It consists of lifeless cells that accumulate in the wound exudate.
Moisture Associated Skin Damage (MASD)	a spectrum of injury characterized by the inflammation and erosion of the epidermis resulting from prolonged exposure to various sources of moisture

	and potential irritants (eg, urine, stool, spontaneous rupture of membranes, perspiration, wound exudate, and ostomy effluent).
Exudate	Fluid that spills from the circulatory system into wounds or areas of inflammation. It can be clear or more yellow in colour
Device Related Pressure Ulcer (DRPU)	A pressure ulcer that has developed due to the presence of a medical device (NHSI June 2018)
Hospital acquired pressure ulcer (HAPU)	A “new” pressure ulcer is one that is first detected within the current episode of care, sustained whilst an inpatient.
Community acquitted pressure ulcer (CAPU)	A pressure ulcer detected when undertaking a skin inspection as part of the admission to the service- Generally Accident & Emergency.
Suspected deep tissue injury (SDTI)	Maroon or purple areas of intact skin or blood-filled blisters caused by harm to the underlying tissues and are known to deteriorate quickly even under expert care
Unstageable Pressure Ulcer	Localised areas of necrosis that characteristically develop when tissue is compressed over a bony prominence and an external surface for a sustained period of time. Ulcers covered with slough or eschar is by definition an unstageable pressure ulcer.

Assessment of skin integrity

Antenatally, women/birthing people may be assessed during the third trimester prior to their admission to the maternity unit. For those women/birthing people who are identified as high risk, using the NICE guidelines, in conjunction with clinical judgement/ assessment and the Purpose T risk assessment tool referrals can be made to for specialist support. An assessment by the manual handling advisor or tissue viability nurse may be planned, so interventions can be implemented ahead of admission including the provision of specialist equipment.

Antenatally prior to admission all women/birthing people need to be informed about the importance of looking after their skin and regular mobilisation/ repositioning, including during labour.

All women/birthing people who are admitted to the unit must have the Purpose T Risk Assessment Tool completed within 6 hours of admission, this includes patients admitted for antenatal care and those admitted in active labour. The risk of pressure ulcers must be reassessed if there is a change in clinical status e.g. epidural sited, prolonged labour, surgical intervention, requires high dependency care or prolonged immobility.

Fully body skin inspection must also be completed within 6 hours of admission, ensuring any skin damage present on admission is documented on a body map (appendix 1), with an incident report completed.

Skin inspection should include assessment for localised heat, oedema, or induration (hardness), especially in individuals with darkly pigmented skin. Use finger palpation to assess erythema or discolouration to determine whether skin is blanchable. The most vulnerable areas should be concentrated on, these include:

- Sacrum
- Ischial Tuberosity
- Femoral trochanters
- Elbows
- Temporal regions of the skull
- Shoulders
- Back of the head
- Ears
- Feet (especially in patients with neuropathy)
- Heels
- Ankles
- Any areas with a medical device

Purpose T Risk assessment Tool

PURPOSE-T is a pressure ulcer risk assessment framework, intended to identify adults at risk of pressure ulcer development and makes a distinction between primary prevention (applicable to those at risk of pressure ulcer development) and secondary prevention (applicable to those who already have a pressure ulcer). It has been developed for use in adult populations in hospital and community settings by registered staff members.

Appendix 2: Purpose-T document

Pressure redistributing devices

Following a completed risk assessment (purpose T), based on an amber or red outcome or clinical judgement, a specialist pressure relieving mattress may be required, this does not replace the need for repositioning/redistributing the woman/birthing person's weight.

Women/birthing people still need to be repositioned as directed by their individual risk assessment. Additional equipment may also be required to prevent or reduces incidences or pressure. All women/birthing people who are managed on air alternating mattresses must be assessed daily to ensure that this is an ongoing requirement. When the alternating air mattress is no longer clinically indicated the woman/birthing person should be transferred to a foam mattress and the alternating air mattress should be returned to the equipment library to be decontaminated and made available for the next patient. If an alternating air mattress is not available at the time of request this needs to be escalated to the senior nurse for the department, to the TVN, it should be documented in the patient notes and an incident report completed. This information must be handed over between shifts to ensure a mattress is sourced at the earliest opportunity.

Within the Trust, we have the following pressure relieving equipment available;

- Specialist pressure relieving mattress (air alternating – equipment library)
- Specialist pressure relieving cushion (air alternating – equipment library)
- Bariatric pressure relieving mattress (air alternating – specialist order)
- Repose boots (heel protectors – local order)
- Repose wedges (offloading device – local order)
- Kerrapro sheets (for intact skin only, not open wounds – local order)
- Specialist immersion therapy (authorised by Tissue Viability – specialist order)
- Slidesheets (local order)

Repositioning

Encourage all women/birthing people, especially those who have been considered as being at risk of developing a pressure ulcer to change their position frequently and at least every 2-4 hours. If they are unable to reposition themselves, offer help to do so, using suitable equipment if needed (slide sheets). Document the regularity of repositioning required within the patient case notes – this needs to be clearly evidenced and once a woman/birthing person has been repositioned, include what position they have now been repositioned into (for example, repositioned from left side, to back), ensure that they are left comfortable in clean beds.

Management of Exposure to Moisture

All women/birthing people should receive regular checks to monitor for exposure to moisture during labour. The use of a barrier product should be considered to prevent skin damage in women/birthing people who are immobile, at high risk of developing moisture associated skin damage or incontinence-associated dermatitis, as identified by skin assessment. All efforts should be made to remove sources of moisture, ensuring skin is not exposed any longer than needed. Barrier films should also be considered for peri wound protection.

Appendix 3: MASD pathway

Referral for specialist Tissue Viability input

The purpose of the Tissue Viability Services is to provide specialist guidance on non-healing and complex wounds, as well as providing education to clinical staff. It is key that referrals to the service are suitable and timely to ensure best outcome for the patient and utilising the Tissue Viability Service and the resources available. This policy includes all patients across the Hospital Trust with Tissue Viability needs. Referrals to the Tissue Viability Service should be either via bleep 7040 (within standard working hours) or be submitted via Hillingdon Care Record with appropriate details to allow effective triage and prioritisation.

Pressure ulcer management

Ulcer dimensions

Document the surface area of all pressure ulcers, length, width and depth. If undermining is present, this should be included in documentation. Disposable tape measures and probes are available on all wards.

Categorisation

Categorise each pressure ulcer using an authenticated classification tool (such as the International NPUAP-EPUAP Pressure Ulcer Classification System Appendix 4). Repeat and document each time the ulcer is assessed and update Governance accordingly via Datix if the pressure ulcer deteriorates further whilst the patient remains on the inpatient unit.

Clinical Photography/Video Procedure

For a comprehensive pressure ulcer assessment and to assist with any RCA clinical photographs must be taken as soon as possible after admission, please contact TVN team to support with this process.

All clinical Photography should be used in accordance with the Trusts consent form. All pressure related wounds, regardless or origin (Hospital acquired, or community) should be photographed and stored on patients (HCR) file.

Dressings

All care plans and proposed dressing regimes should be discussed with the woman/birthing person, if appropriate, their family or carers, when deciding which type of dressing should be used, the following should be taken into account:

- Pain and tolerance
- Location of the ulcer
- Exudate level
- Frequency of dressing change.

Dressings promote a warm, moist wound healing environment to treat category 2, 3 and 4 pressure ulcers. Category 1 pressure ulcers should not be dressed (intact skin does not required a dressing). Referral to community services for ongoing support should be made in conjunction with advice from the hospital TVN team to ensure that the correct pathway is followed.

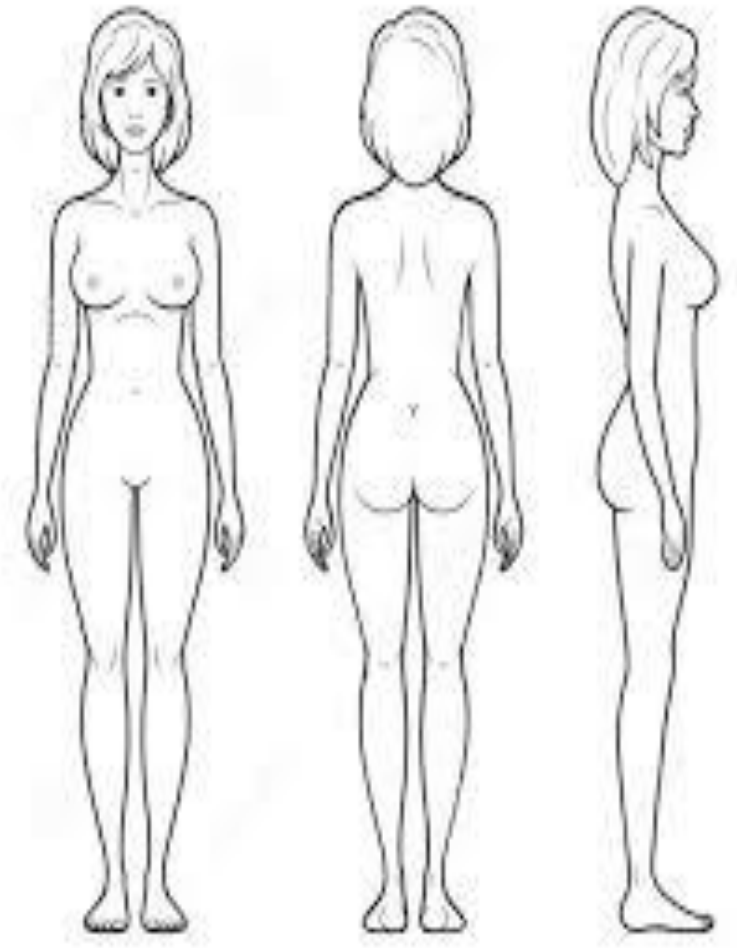
Reporting pressure ulcers/ injuries

Pressure ulcers category 2, 3, 4, Suspected Deep Tissue Injury (SDTI), unstageable, Moisture associated skin damage (MASD), Pressure ulcer on admission and device related pressure ulcer, must be reported to Clinical Governance through the trust incident reporting framework. Safeguarding referrals must be considered as part of this process using the safeguarding decision support tool (Appendix 5).

The Clinical Governance Team provides a weekly pressure ulcer summary report to the Director of Nursing and senior nurses / midwives within the trust. Hospital acquired pressure ulcers (ward by ward) are discussed at the senior nurse/midwife meeting. Pressure Ulcer incidents are involved in the Trusts Integrated Quality and Performance Report. Divisional Pressure Ulcers are monitored and discussed at the Maternity Governance Group.

Appendix 1: Maternity Body Map Tool

Maternity Body Map Tool	
Patient label **Hospital No** **Patient Name** **DOB** **Ward**	Please complete body map for all patients within 6 hours of admission to hospital Skin inspection should include assessment for localized heat, oedema, or induration (hardness), especially in individuals with darkly pigmented skin



Please use the body map to annotate any areas of skin damage and then record into the patient notes in detail.
If any pressure ulcers are identified, please complete an incident report, escalate to the senior midwife on duty and refer to the Tissue Viability Nurses for support.

Appendix 2 – Purpose T

Pressure Ulcer Risk Assessment – PURPOSE T (V2)

Patient name	DOB	Hospital / NHS number	Ward

Step 1 – screening

Mobility status – *tick all applicable*

- Needs the help of another person to walk
- Spends all or the majority of time in bed or chair
- Remains in the same position for long periods
- Walks independently with or without walking aids

If **ONLY** blue box is ticked

Skin status – *tick all applicable*

- Current PU category 1 or above?
- Reported history of previous PU?
- Vulnerable skin
- Medical device causing pressure/shear at skin site e.g. O_2 mask, NG tube
- Normal skin

If **ONLY** blue box is ticked

Clinical Judgment – *tick as applicable*

- Conditions/treatments which significantly impact the patient's PU risk e.g. poor perfusion, epidurals, oedema, steroids
- No problem

If **ONLY** blue box is ticked

No pressure ulcer **not currently at risk**

Tick if applicable

Not currently at risk pathway

If **ANY** yellow boxes are ticked, **go to Step 2**

If **ANY** yellow or pink boxes are ticked, **go to Step 2**

If **ANY** yellow boxes are ticked, **go to Step 2**

Step 2 – full assessment

Complete ALL sections

Analysis of independent movement

Tick the applicable box (where frequency and extent categories meet)

Frequency of position changes	Extent of all independent movement – Relief of all pressure areas: Doesn't move	Slight position changes	Major position changes
Doesn't move		N/A	N/A
Moves occasionally	N/A		
Moves frequently	N/A		

Sensory perception and response – *tick as applicable*

- No problem
- Patient is unable to feel and/or respond appropriately to discomfort from pressure e.g. CVA, neuropathy, epidural

Moisture due to perspiration, urine, faeces or exudate – *tick as applicable*

- No problem / Occasional
- Frequent (2–4 times a day)
- Constant

Diabetes – *tick as applicable*

- Not diabetic
- Diabetic

Perfusion – *tick all applicable*

- No problem
- Conditions affecting central circulation e.g. shock, heart failure, hypotension
- Conditions affecting peripheral circulation e.g. peripheral vascular / arterial disease

Nutrition – *tick all applicable*

- No problem
- Unplanned weight loss
- Poor nutritional intake
- Low BMI (less than 18.5)
- High BMI (30 or more)

Medical device – *tick as applicable*

- No problem
- Medical device causing pressure/shear at skin site e.g. O_2 mask, NG tube

Vulnerable skin (precursor to PU) e.g. blanchable redness that persists, dryness, paper thin, moist.
NPUAP / EPUAP Pressure Ulcer Classification System (2009)
Cat 1 Non-blanchable redness of intact skin
Cat 2 Partial thickness skin loss or clear blister
Cat 3 Full thickness skin loss (fat visible/ slough present)
Cat 4 Full thickness tissue loss (muscle/bone visible)
Cat U (Unstageable/Unclassified): full thickness skin or tissue loss - depth unknown

Current Detailed Skin Assessment – *tick if pain, soreness or discomfort present at any skin site as applicable. For each skin site tick applicable column – either vulnerable skin, normal skin or record PU category*

Skin site	Pain	Vulnerable skin	PU category	Normal skin
Sacrum				
L Buttock				
R Buttock				
L Ischial				
R Ischial				
L Hip				
R Hip				
L Heel				
R Heel				
L Ankle				
R Ankle				
L Elbow				
R Elbow				
Other as applicable (may be medical device site)				

Previous PU history – *tick as applicable*

- No known PU history
- PU history – *complete below*
- Number of previous pressure ulcer(s)

Detail of previous PU (if more than 1 previous PU give detail of the PU that left a scar or worst category).

Approx date	Site	PU cat	Scar	No scar

Other relevant information (if required):

Step 3 – assessment decision

If **ANY pink boxes** are ticked/completed, the patient has an existing pressure ulcer or scarring from previous pressure ulcer.

If **ANY orange boxes** are ticked (but no pink boxes), the patient is at risk.

If **only yellow and blue boxes** are ticked, the nurse must consider the risk profile (risk factors present) to decide whether the patient is at risk or not currently at risk.

PU Category 1 or above or scarring from previous pressure ulcers

Tick if applicable

Secondary prevention and treatment pathway

No pressure ulcer but **at risk**

Tick if applicable

Primary prevention pathway

No pressure ulcer **not currently** at risk

Tick if applicable

Not currently at risk pathway

Nurse printed name	Nurse signature	Date	Time

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Appendix 3 – Maternity Moisture Associated Skin Damage Flow Chart

Is the patient at risk of prolonged exposure to fluid?
(Including prolonged exposure to ruptured membrane and Urinary Incontinence)

YES

- Consider is the patient suitable for a catheter if immobile or has altered sensation?
- Complete/ Update the relevant risk assessment's (e.g. Purpose T)

REMEMBER eliminating the moisture where possible is crucial for prevention/management – ensure the patient is as clean / dry as possible – regular review of the incontinence pad is key

Has all the above been considered and patient remains high risk?

NO

Monitor closely for skin breakdown.
Consider the use of a prophylactic barrier cream;
Medi Derma S – Total Barrier C

On reassessment if patient becomes incontinent of urine or stool, or any changes to membranes repeat the question above.

If skin damage/breakdown is noted and ward needs additional support or advice please refer to Tissue Viability Services via HCR (Referral guidance available on intranet).
TVN Bleep 7040 **TVN Ext 3824**

YES

-Use Derma S Spray/Cream Three times a day
-Use 3M Cavillon Lollipop after each episode of personal care
-Avoid using wet wipes and ensure skin is dried thoroughly, sheets should be changed regularly
- encourage patient to use commode/ toilet where able
-If no improvement upgrade to the Medi-Derma pro 2 step foam and spray incontinence cleanser
-Ensure all actions are clearly documented

Derma S Creams, and Cavilon Sprays and Lollipops are all available through Stores/ ONPOS.

No

-Provide medi derma S cream for patient to use minimum of BD
-Encourage use of dry wipes and ensure skin is dried thoroughly
-Ensure staff and patient communicate regarding skin integrity and document this clearly in your patient notes

Appendix 4 – EPUAP Classification

https://www.epuap.org/

Pressure Ulcer Categorisation Guide

How to Categorise[1]

Category I		Intact skin with localised non-blanchable erythema, usually over a bony prominence. Skin discolouration, warmth, oedema, hardness or pain may be present and may differ compared to adjacent tissue. May indicate "at risk" persons.	
Category II		Partial thickness loss of dermis presenting as a shiny or dry, shallow open ulcer with a red/pink wound bed with minimal slough or bruising. Can also include intact or ruptured blisters. Does not include skin tears, tape burns, incontinence associated dermatitis, maceration or excoriation.	
Category III		Full thickness tissue loss. Subcutaneous fat may be visible but bone, tendon or muscle are not exposed or directly palpable. Some slough may be present. May include undermining and tunnelling. Depth varies by location and can be shallow in areas without subcutaneous tissue eg. on the ear, or extremely deep in areas of significant adiposity.	
Category IV		Full thickness tissue loss with visibly exposed or directly palpable bone, tendon or muscle. Often includes undermining and tunnelling. Depth varies by location and can be shallow in areas without subcutaneous tissue eg. on the ear. Can extend into muscle and/or supporting structures (eg. fascia, tendon or joint capsule) making osteomyelitis likely to occur.	
Unstageable/ Unclassified		Full thickness tissue loss where actual ulcer depth is obscured by slough (yellow, tan, grey, green or brown) and/or eschar (tan, brown or black). True depth cannot be determined until enough slough and/or eschar is removed, but will be either a Category III or IV. Stable (dry, adherent, intact without erythema or fluctuance) eschar on the heel should not be removed until a Doppler assessment has been completed.	
Suspected Deep Tissue Injury (SDTI)		Purple or maroon localized area of discoloured intact skin or blood-filled blister due to pressure/shear damage of underlying soft tissue. May be preceded by painful, firm, boggy, warmer or cooler skin as compared to adjacent tissue. Evolution may be rapid, exposing additional tissue layers even with treatment.	

Based on International NPUAP-EPUAP-PPPIA Pressure Ulcer Classification System.[1]
NB: Injuries may be more difficult to detect in more darkly pigmented skin, and may not have visible blanching; Its colour may differ from the surrounding area.

Reference: 1. NPUAP, EPUAP and Pan Pacific Pressure Injury Alliance. Prevention and Treatment of Pressure Ulcers: Quick Reference Guide. Emily Haesler (Ed.). Cambridge Media: Osborne Park, Western Australia; 2014.

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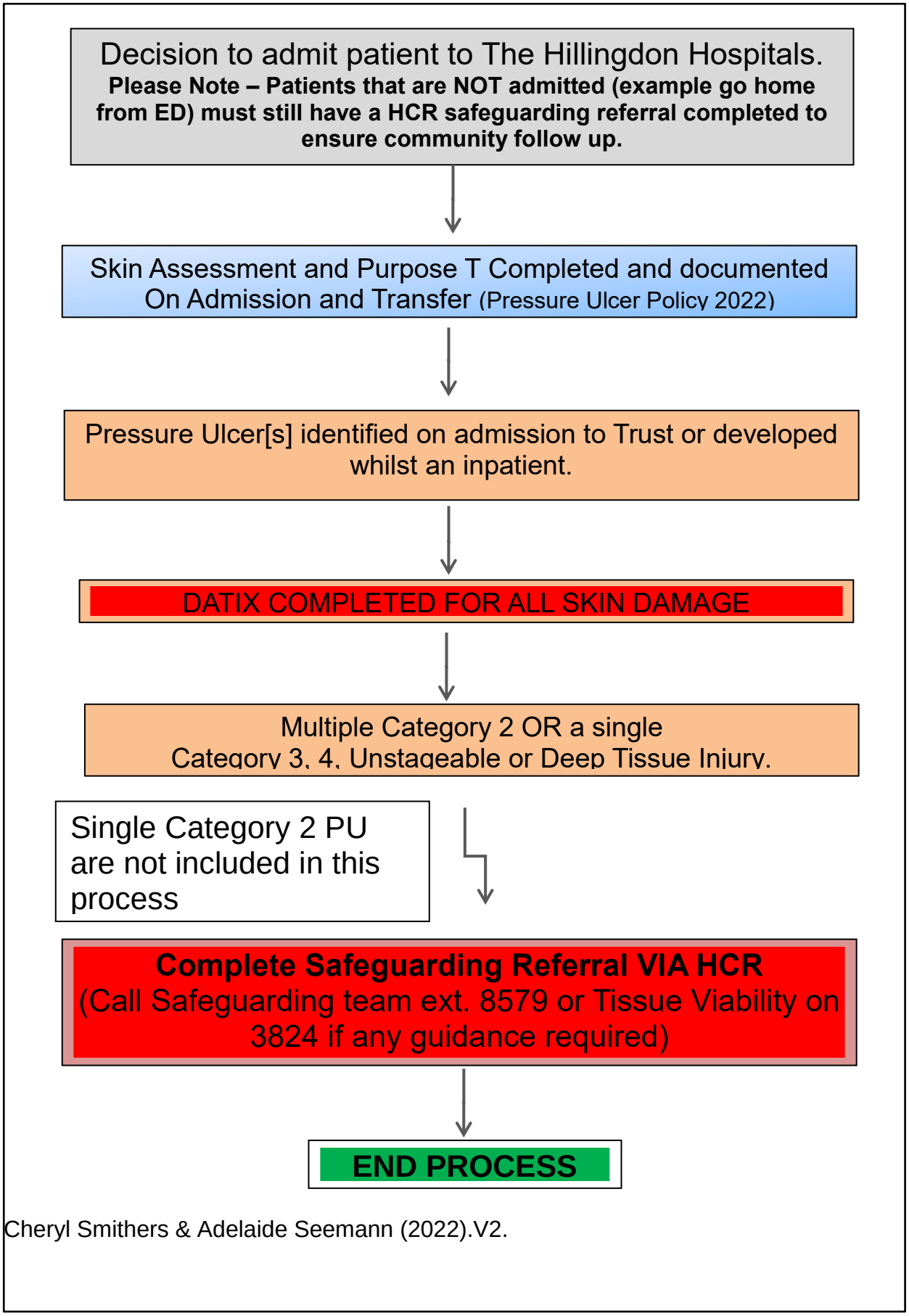
Appendix 5 – Safeguarding Adults Decision Tool for Patients with Pressure Ulcers



Cheryl Smithers & Adelaide Seemann (2022).V2.